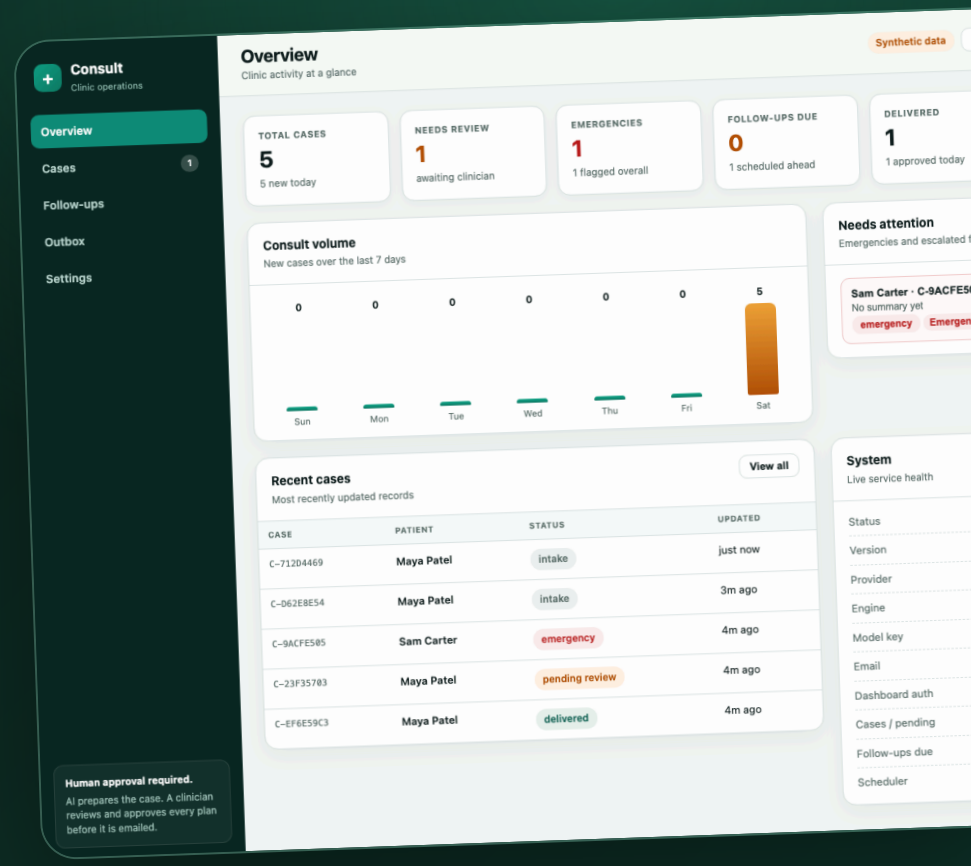
Consult Homeopathic Physicians

AI intake. **Doctor decides.**

A safe, multi-step clinic workflow: conversational intake by text or voice, deterministic emergency screening, document reading, structured case preparation, human approval, email and follow-up escalation.



Live on Vultr consult-homeopathic.207.148.72.102.sslip.io

Synthetic demo data. Not a medical device.

Intake work steals clinical attention.

WHO FEELS IT

A small clinic with no intake team

The clinician, front desk and practice owner all handle the same new-patient story across phone, chat, paper and inbox.

THE MOMENT

A lab PDF, a waiting patient, a red flag

Symptoms, history, allergies, medications and lab values arrive unstructured. The urgent case competes with the routine paperwork.

THE EXPENSIVE FAILURE

A red flag buried in the story

Missed escalation is the failure that matters. Speed without a safety gate is not a product.

The doctor should spend time **deciding**, not retyping the patient story.

One case record. Two workflows. One hard gate.

PATIENT

Talk, attach, choose

A short text or voice conversation, an optional text/CSV/PDF document and two or three plain-language care paths.

→

AGENT

Screen, structure, prepare

Deterministic red-flag gate, structured history, document parsing, summary, safety warnings and a follow-up plan.

→

DOCTOR

Review, edit, approve

The operations console shows the case, documents, prior visits, conversation and audit trail. Approval is the only path that sends a plan.

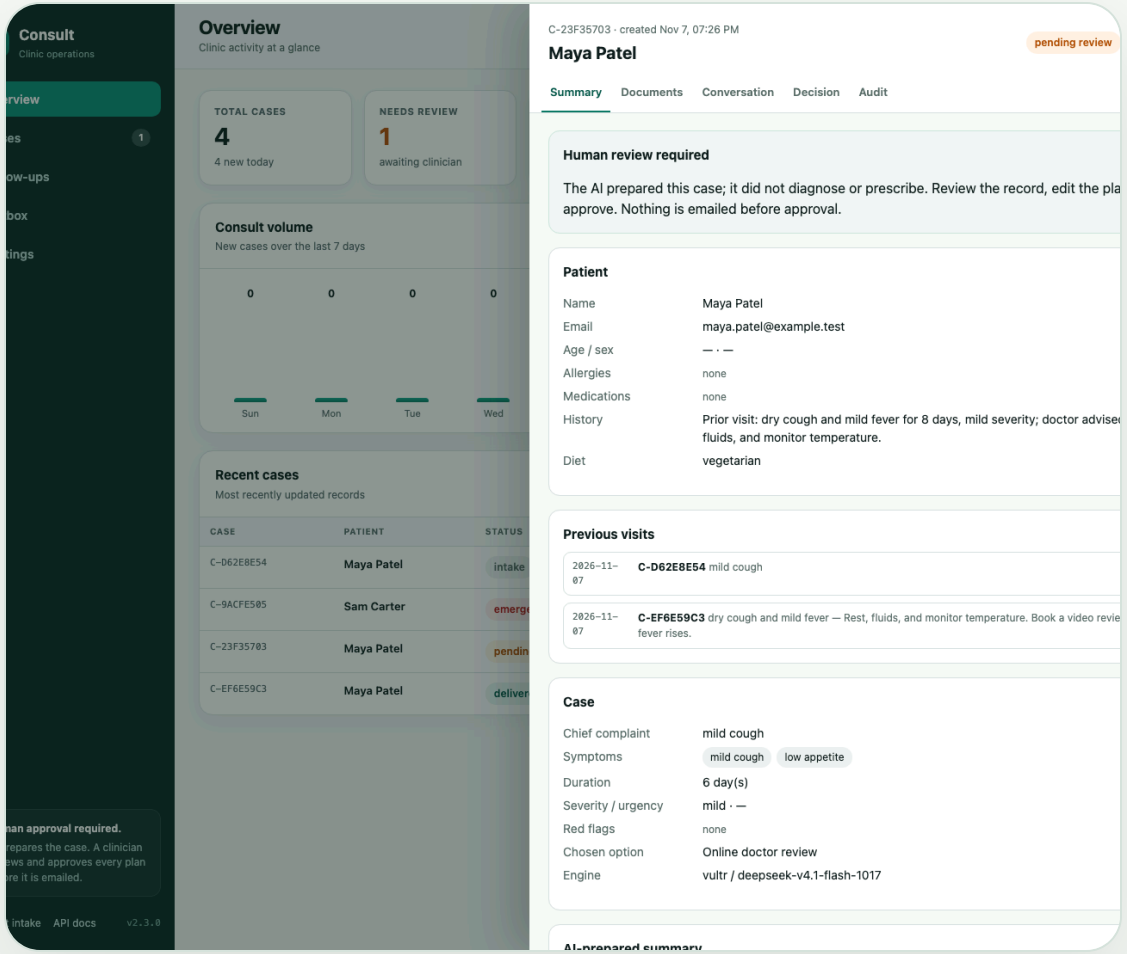
HUMAN APPROVAL IS A HARD GATE The AI never diagnoses and never prescribes. Every approval is single-use and audited.

The golden path, running now.

Open the public app, then sign in to the clinic console. The whole loop takes about two minutes.

1. Complete a synthetic intake by text or voice and choose a care option.
2. Sign in at **/doctor** and open the pending case.
3. Inspect the summary, documents, allergy warning and conversation; approve a plan.
4. See the plan in Outbox, then submit a red-flag follow-up and watch the case escalate.

Live app: **consult-homeopathic.207.148.72.102.sslip.io**
Dashboard: **/doctor** · user **doctor** · password in submission notes



Vultr is the control plane.

PATIENT BROWSER

Intake + follow-up

Public pages, no staff credential.

DOCTOR BROWSER

Operations console

Signed session, staff APIs only.

VULTR CLOUD COMPUTE

FastAPI control layer

Case state · deterministic safety gate · approval gate · audit trail · SQLite · documents · email · scheduler

Caddy TLS · systemd

VULTR SERVERLESS INFERENCE

Language model

OpenAI-compatible endpoint; the key stays server-side.

VERIFIABLE OUTPUT

Plan + audit + follow-up

Email outbox, token check-in and escalation state.

Model drafts. The application owns state, safety and approval. If inference is unavailable, a labelled rule fallback keeps the workflow usable.

Safety is structural, not a prompt.

GATE 01

Red flags before the model

A deterministic, negation-aware rule gate runs before every model call. The model can add a flag; it cannot override the gate.

GATE 02

Single-use human approval

Only **/approve** sends a plan. It requires pending review, cannot run twice and is blocked for emergency cases.

GATE 03

No AI diagnosis or prescribing

The agent prepares the case. Prescription text is entered by the doctor; a reply guard blocks explicit dosing instructions.

GATE 04

Everything leaves a trail

Patient, agent, safety and doctor actions are timestamped. Every reply records its engine, including labelled rule fallbacks.

Start with the clinics that feel the pain.

Small independent clinics and telehealth teams with 1–15 clinicians. They need doctor oversight but cannot staff a full intake and documentation team.

220k–260k

US physician practices

~73%

of physician offices have fewer than 50 employees

$190M–$225M

US-only annual software ceiling at $99/clinic/month

Bottom-up: ~160k–190k small practices × $99/month × 12 ≈ $190M–$225M ARR. Expansion: telehealth, multi-clinic groups and English-speaking markets. This is a planning assumption, not a forecast.

Sources: US Census SUSB analysis 2025; AMA/PMC practice size

Simple SaaS, aligned with clinic value.

STARTER

$99/clinic/month

1–2 clinicians, intake, case queue and follow-ups.

CLINIC

$199/clinic/month

Up to 5 clinicians, outbox, audit export and priority support.

TELEHEALTH

$299+ usage

Higher volume, custom intake forms and integration work.

> **ROI scenario:** save 10 minutes per new patient × 20 new patients/week ≈ 3.3 staff hours/week, before counting fewer missed follow-ups and a complete audit trail. The buyer is the practice owner; the daily users are the clinician and front desk.

Why this needs AI, not another form.

WHAT THE MODEL DOES

Meet the patient in their own words

- Understands free-form symptoms, history and medication context.
- Chooses the next question instead of forcing a fixed form.
- Extracts a structured case and reads unstructured documents.
- Drafts a summary and safe care options for the doctor.

WHAT IT NEVER DOES

No diagnosis. No prescription. No approval.

- The deterministic safety gate blocks emergencies first.
- The doctor edits and approves the plan.
- No plan is sent without human approval.
- No API key reaches the browser.

Why now: capable language models + Vultr Serverless Inference make this deployable without operating a GPU stack. The model is load-bearing, but the control layer remains deterministic.

Next: pilot with real clinics.

BUILT BY

Canh Pham

Backend, inference integration, workflow design, dashboard and Vultr deployment. Solo build for the Vultr Agent Rush Hackathon.

Live app | /doctor console | 76 automated tests

NEXT 3 MILESTONES

01 · Pilot

2–3 clinic design partners with synthetic and consented data.

02 · Integrate

SMTP, calendar and EHR/fax handoff for real operations.

03 · Harden

Per-user auth, consent, encryption at rest and compliance review.

Ask: pilot clinics and clinical feedback. **Live:** consult-homeopathic.207.148.72.102.sslip.io · **Source:** github.com/pham-canh/consult-homeopathic